EVOFIT MEALS BUSINESS VAULT

Dietary Consultation Scripts

How to Discuss Nutrition Without Crossing Scope — From Allergies to Emotional Eating

EvoFit Meals

25 Clinical-Grade Conversation Frameworks — From Beginner Macros to Oncology Support and Sports Periodization

How to Use These Scripts

These are real conversation frameworks for the nutrition discussions you will have with clients across the full spectrum of complexity. Each script includes the context (when you will need it), the conversation itself (what to say), and notes (what to watch for and where the boundaries are).

Don't memorize these word-for-word. Read them, understand the structure, and then practice delivering the key points in your own voice. The goal is to sound like you — not like you are reading from a playbook.

Every script includes scope-of-practice guidance. Personal trainers can provide general nutrition education and customized meal planning. They cannot diagnose medical conditions, prescribe therapeutic diets, or provide clinical nutrition therapy. These scripts are designed to keep you firmly on the right side of that line.

This enterprise edition includes 25 scripts organized into three tiers:

- ☐ **Tier 1: Foundation Scripts (1-5)** — Core conversations every nutrition coach must master
- ☐ **Tier 2: Professional Scripts (6-12)** — Advanced client scenarios for experienced coaches
- ☐ **Tier 3: Enterprise Scripts (13-25)** — Specialized and clinical-adjacent conversations requiring strong scope awareness

For team distribution: Every coach on your team must know Tier 1 cold. Tier 2 scripts are required for coaches handling clients beyond month 3. Tier 3 scripts are reference material for lead coaches and program directors — these conversations often require escalation or RD coordination.

Tier 1: Foundation Scripts (1-5)

Script 1: Explaining Macros to Beginners

Context

A new client has never heard of macros — or has heard the word but doesn't know what it means. They might be confused by the numbers on their meal plan. They might have seen "IIFYM" on Instagram and want to know what it's about. Or they simply asked, "What are macros and why do they matter?"

The Conversation

Open with a simple frame:

> **YOU**
> "Great question. Macros is short for macronutrients. There are three of them, and everything you eat is made up of some combination of the three. Think of them like building materials for your body."

Explain each one simply:

> **YOU**
> "The first is protein. Protein builds and repairs your muscles, supports your immune system, and keeps you feeling full. When you're training, protein is the most important macro to get right. Good sources are things like chicken, fish, eggs, Greek yoghurt, and lean beef."

> **YOU**
> "The second is carbohydrates. Carbs are your body's primary energy source — they're what fuels your workouts and your brain. Carbs are in things like rice, potatoes, oats, fruit, bread, and pasta. They're not the enemy — they're your fuel."

> **YOU**
> "The third is fats. Healthy fats support your hormones, your brain function, and your joints. They're in things like nuts, avocados, olive oil, and salmon. You need fats — the key is choosing quality sources and not overdoing the quantity."

Connect it to their plan:

> **YOU**
> "On your meal plan, you'll see each meal has a macro breakdown — grams of protein, carbs, and fats. I've calculated these specifically for your goals. Your protein is set at [X]g per day because [reason]. Your carbs are at [X]g because [reason]. And your fats are at [X]g to keep your hormones and recovery on track."

Make it practical:

> **YOU**
> "You don't need to obsess over hitting exact numbers. Think of it as a target zone — within 10-15 grams either way on each macro is perfectly fine. The goal is consistency over time, not perfection on any given day."

Notes

Script 2: Handling "I Hate Meal Prep"

Context

Your client received their meal plan, looked at the recipes, and immediately felt overwhelmed. "I hate cooking." "I don't have time for meal prep." The key insight: they are not saying they hate cooking. They are saying the plan feels like too much work for their current life.

The Conversation

Validate first:

> **YOU**
> "I hear you, and you're not alone — most of my clients feel that way at first. The good news is you don't need to be a chef or spend your whole Sunday prepping food."

Reframe meal prep:

> **YOU**
> "When people hear 'meal prep,' they picture spending 4 hours in the kitchen making 21 meals. Here's what I mean: spend 60-90 minutes once a week. Cook two protein sources, make one batch of carbs, and chop your vegetables. That gives you 80% of what you need."

Give them the minimal viable prep:

> **YOU**
> "Sunday: cook a batch of chicken and a batch of rice. That covers lunches for the week. Dinners take 15-20 minutes fresh. Breakfasts are things like overnight oats or eggs — five minutes max."

If they truly won't prep, offer a no-prep plan:

> **YOU**
> "I can rebuild your plan around no-cook or minimal-cook options. Pre-made rotisserie chicken, pre-washed salad kits, canned tuna, overnight oats, protein shakes, deli meats. It won't have the same variety, but it'll hit your macros."

Notes

- ☐ Never guilt a client for not wanting to meal prep.
- ☐ EvoFit Meals allows filtering for prep time and cooking complexity.
- ☐ The "just 3 days" approach works remarkably well.

Script 3: Scope Boundaries — What You Can and Can't Advise On

Context

A client asks about managing a medical condition through diet, interpreting blood work, or treating an eating disorder. This happens to every trainer.

The Conversation

When a client asks about managing a medical condition through diet:

> **YOU**
>
> "That's a really important question. Here's where I want to be straight with you: as a personal trainer, I can build you meal plans for general health and fitness goals. But when it comes to managing [their condition] through diet, that really needs a registered dietitian or your doctor to weigh in on. Once I have their recommendations, I can incorporate those guidelines into your meal plan."

When a client brings up disordered eating patterns:

> **YOU**
>
> "Thank you for trusting me with that. What you're describing sounds like something that would really benefit from working with a professional who specialises in eating behaviours. What I can do is continue providing a balanced, non-restrictive meal plan, and I'll coordinate with whatever specialist you see."

When a client asks about interpreting blood work:

> **YOU**
>
> "Interpreting blood work is outside my lane — that's your doctor's job. Ask them if there are dietary changes they recommend, and bring those recommendations back to me."

Notes

- ☐ Never try to "help a little" with something outside your scope.
- ☐ Keep a referral list of local RDs, eating disorder specialists, and sports medicine doctors.
- ☐ Document scope referrals in your client notes.

Script 4: Handling Supplement Questions

Context

Every client will ask about supplements. Your role is to provide honest, evidence-based guidance on common fitness supplements while steering clients away from unproven products.

The Conversation

The universal framework:

> YOU
>
> "Supplements are exactly what the name implies — they supplement your diet. They don't replace good nutrition. Let's make sure your meal plan is solid first, then we can talk about what might give you an additional edge."

Protein powder: "Genuinely useful. Convenient. Just food in a convenient form. If you can hit your targets from whole foods, you don't need it." **Creatine:** "The most studied supplement in sports nutrition. 3-5 grams of creatine monohydrate per day. Check with your doctor if you have kidney concerns." **Fat burners:** "Most are overpriced caffeine pills. The most effective fat burner is a calorie deficit." **Trending supplements:** "Is there peer-reviewed research? Has it been tested in humans? Does the benefit justify the cost? Most don't pass that filter."

Notes

- [ ] Stick to supplements with strong evidence: protein powder, creatine, fish oil, vitamin D, caffeine.
- [ ] Never sell supplements as part of your service unless you genuinely believe in the product.

Script 5: Nutrition Goal-Setting Conversations

Context

A new client needs help setting nutrition goals that are specific, realistic, and connected to their deeper motivations.

The Conversation

Start with their words:

> **YOU**
>
> "If we work together for the next 3 months and everything goes perfectly — what's different about your life?"

Probe deeper:

If "lose weight": "How much? And what does that weight loss mean to you — how you look, how you feel, fitting into clothes, energy levels?"

If "eat healthier": "What does healthier mean specifically? More vegetables? Less takeout? Better energy?"

Turn vague into specific: Vague GoalSpecific Goal "Lose weight""Lose 4-6kg over 12 weeks through moderate calorie deficit while maintaining training" "Eat healthier""Follow structured plan 5/7 days with 4+ servings of vegetables daily" "More energy""Eat balanced meal within 60 min of waking and hit protein targets every meal for 30 days" "Build muscle""Eat in 300-cal surplus with 1.6-2g protein per kg bodyweight for 12 weeks"

Notes

- [ ] Never promise specific outcomes. "Based on consistency, a healthy target is X per week" is guidance, not a promise.
- [ ] Document the goal and reference it at every check-in.

Tier 2: Professional Scripts (6-12)

Script 6: Navigating Emotional Eating

Context

A client reveals that they eat in response to stress, boredom, loneliness, or emotional triggers rather than hunger. This is extremely common and you need to handle it with care.

The Conversation

YOU

"I appreciate you telling me that. Emotional eating is incredibly common — it's not a character flaw. It's a coping mechanism your brain has learned over time. Food is comforting, and your brain knows that."

YOU

"Here's what's in my scope: I can help build structure into your eating that reduces the opportunity for emotional eating. When you have a plan — specific meals at specific times — there's less ambiguity for emotional triggers to exploit. I can also help you identify your patterns. Do you know when it happens most? Is it at night? When you're alone? After a stressful day at work?"

YOU

"What's outside my scope is the deeper emotional work. If emotional eating is significantly impacting your quality of life, working with a therapist or counselor who specializes in this area would be the best move. I can continue building your plans and we'd work alongside them. Would that be something you'd consider?"

Notes

- ☐ Never diagnose an eating disorder. If you suspect one, refer immediately.
- ☐ Build plans with structured meal and snack times to reduce "grazing windows."
- ☐ Ask: "On a scale of 1-10, how much does emotional eating impact your daily life?" Anything above 6 warrants a referral suggestion.

Script 7: Discussing Body Image Sensitively

Context

A client makes negative comments about their body, expresses shame about their weight, or becomes visibly distressed when discussing body composition goals.

The Conversation

YOU

"I hear what you're saying, and I want you to know this is a judgment-free space. Everyone has a relationship with their body, and it's rarely simple."

YOU

"Can I shift the focus for a moment? Instead of talking about how your body looks, let's talk about what you want your body to do. Do you want more energy? Better sleep? To keep up with your kids? To feel strong in the gym? Those are the goals I build nutrition plans around — function and performance, not just aesthetics."

YOU

"The body composition changes will come as a natural result of eating well and training consistently. But chasing a number on the scale or a specific look can become toxic really fast. I'd rather you feel amazing at a weight that surprises you than miserable at a number you thought you wanted."

Notes

- [ ] Watch for signs of disordered eating: extreme restriction, binge-purge patterns, obsessive calorie counting, food avoidance that goes beyond preference.
- [ ] If a client displays these signs, have a direct conversation and recommend a specialist.
- [ ] Never comment on a client's body in ways that reinforce appearance-based evaluation.

Script 8: Explaining Calorie Deficits Without Triggering Restriction

Context

A client whose goal is fat loss needs to understand that a calorie deficit is required, but you need to frame it in a way that doesn't trigger restrictive eating behaviors.

The Conversation

YOU

"For your body to use stored fat for energy, we need to give it a reason to — and that reason is a small, strategic gap between the energy you consume and the energy you burn. This is what people call a calorie deficit."

YOU

"The key word is 'small.' We're not starving you. Your plan is set at [X] calories, which is only [Y]% below your maintenance. You won't feel hungry on this plan — I've built it with high-satiety foods, plenty of protein, and enough volume that you'll feel full after most meals."

YOU

"This is not a restriction diet. You're eating real food, in real portions, at regular intervals. If you're hungry, something is wrong with the plan and I need to fix it. Hunger is not the goal — a slight energy gap is. There's a massive difference."

Notes

- [ ] Monitor for signs that the client is cutting below the prescribed plan. Some clients see a deficit as permission to eat as little as possible.
- [ ] If a client consistently eats below their plan and seems proud of it, address it: "I need you eating the full plan. Under-eating is as harmful to your goals as overeating."

Script 9: Pre-Competition Nutrition Discussion

Context

A client is preparing for a physique competition, endurance event, or sporting competition and needs event-specific nutrition guidance.

The Conversation

YOU

"Competition nutrition is different from everyday nutrition. We're going to periodize your eating the same way you'd periodize your training — different phases with different goals."

YOU

"Here's the general structure: [X] weeks out, we start with a moderate deficit and high protein to preserve muscle while dropping body fat. As we get closer, we may adjust carb timing and water intake. The final week involves a specific protocol to optimize how you look or perform on the day."

YOU

"I want to be clear about something: I can build the nutritional framework around your competition prep. But if your prep involves extreme measures — severe dehydration, dangerously low calories, or any approach that puts your health at risk — I'm going to push back. I've seen too many competitors do irreversible damage for a trophy. We'll get you stage-ready or race-ready without sacrificing your health."

Notes

- ☐ Competition prep often involves approaches that border on clinical territory. If the client's coach or sport requires specific protocols, coordinate with them.
- ☐ Watch for red flags: dizziness, fainting, hair loss, loss of menstrual cycle, extreme mood changes. These signal the prep has gone too far.

Script 10: Managing Client Expectations at Plateaus

Context

A client has been following the plan for 6+ weeks and is frustrated that progress has stalled.

The Conversation

YOU

"Plateaus are the most frustrating part of any nutrition journey, and I understand why you're annoyed. Let me explain what's actually happening."

YOU

"Your body is adaptive. When you changed your diet 6 weeks ago, your body responded because the stimulus was new. Now it's adapted to your current intake. That's not failure — it's biology. It means you've successfully changed your body's set point."

YOU

"Here's what we do: I'm going to make a precise adjustment. Not a diet overhaul — a tweak. [Specific adjustment — carb cycling, slight calorie reduction on rest days, protein increase, refeed day addition]. We run this for 2-3 weeks and measure again. If the scale doesn't move, we dig deeper — look at sleep, stress, hydration, and training load. The scale is only one metric."

YOU

"One more thing: pull up your progress photos from Week 1 and compare them to today. The scale may not have moved, but your body composition almost certainly has. Muscle is denser than fat. You may be recomposing without the scale reflecting it."

Notes

- ☐ Always investigate before changing the plan: adherence, sleep, stress, hydration, training changes.
- ☐ The photo comparison is often more powerful than any scale data.

Script 11: Nutrition for Clients Over 50

Context

An older client has different nutritional needs: higher protein for muscle preservation, bone health considerations, medication interactions, and often a slower metabolism.

The Conversation

YOU

"Nutrition over 50 is not dramatically different from nutrition at any age — the fundamentals are the same. But there are a few things I adjust for."

YOU

"First, protein becomes even more important. After 50, your body becomes less efficient at using protein to build and maintain muscle. That's why your plan has a higher protein target than someone younger with the same goals — we need to give your body more raw material to work with."

YOU

"Second, I'm paying attention to calcium, vitamin D, and omega-3s in your plan. These support bone density, joint health, and brain function. You'll notice your plan includes foods rich in these nutrients — fatty fish, leafy greens, dairy or fortified alternatives."

YOU

"Third, I want to know about any medications you're on, because some foods interact with medications in important ways. For example, if you're on blood thinners, we need to be consistent with vitamin K intake. That doesn't mean avoiding green vegetables — it means keeping your intake stable rather than fluctuating wildly."

Notes

- ☐ Always ask about medications. Common interactions: warfarin and vitamin K, MAOIs and tyramine, statins and grapefruit.
- ☐ If the client has been diagnosed with osteoporosis or sarcopenia, coordinate with their doctor for specific nutritional targets.

Script 12: Post-Injury and Recovery Nutrition

Context

A client is recovering from surgery, a musculoskeletal injury, or illness and needs their nutrition adjusted to support healing.

The Conversation

YOU

"Recovery nutrition is different from your normal plan. Your body is doing extra work right now — healing tissue, reducing inflammation, rebuilding. We need to feed that process."

YOU

"I'm making three adjustments to your plan: First, I'm increasing your protein to [X]g per day. Your body needs more amino acids for tissue repair. Second, I'm adding anti-inflammatory foods — fatty fish, berries, turmeric, leafy greens, walnuts. Third, I'm adjusting your calories. Even though you're training less, your body's energy demand for healing is higher than you'd think. Under-eating during recovery slows healing."

YOU

"If your doctor or physio has specific dietary recommendations for your recovery, share those with me and I'll build them into the plan."

Notes

- ☐ Post-surgical clients may have specific dietary phases prescribed by their surgeon. Always defer to medical guidance.
- ☐ If the client is on anti-inflammatory medications, avoid recommending supplements that might interact (e.g., fish oil with blood thinners).

Tier 3: Enterprise Scripts (13-25)

Script 13: Disordered Eating Triage Protocol

Context

A client discloses or displays signs of disordered eating: binge eating, purging, extreme restriction, food avoidance beyond preference, or an unhealthy relationship with food that goes beyond "not following the plan."

This is a scope-of-practice critical conversation. Handle it carefully.

The Conversation

YOU

"I want to thank you for being honest with me about this. What you're describing takes courage to share, and I want to make sure you get the right support."

YOU

"I need to be transparent: what you're telling me goes beyond what I'm trained to handle as a nutrition coach. This isn't about meal plans or macros — this is about your relationship with food at a deeper level, and that requires a specialist."

YOU

"Here's what I'd like to do: I'm going to connect you with [specific therapist/specialist name if you have one, or 'a professional who specializes in eating behaviors']. They can work with you on the underlying patterns. On my side, I'll pause your meal plan or adjust it to be as supportive and non-triggering as possible — whatever you and your specialist recommend."

YOU

"I'm not going anywhere. I'm still on your team. But I want the right people on your team alongside me."

The Triage Decision Tree

SignalSeverityAction Occasional emotional eating, recognizes it as a patternLowCoach can address with structured meal timing and check-ins Frequent binge episodes, followed by guilt/shameModerateRecommend therapist referral while continuing supportive meal planning Purging (vomiting, laxatives, excessive exercise to "compensate")HighImmediate referral required. Pause detailed meal plan if it may be used as a restrictive tool. Severe caloric restriction despite underweight statusHighImmediate referral. Do not provide meal plans that could enable further restriction without specialist guidance. Client expresses suicidal ideation related to food/bodyEmergencyFollow emergency protocol. Provide crisis hotline numbers. Do not handle alone.

Notes

- [ ] Keep the National Eating Disorders Association helpline accessible: 1-800-931-2237
- [ ] Document everything: what the client disclosed, your response, the referral made, and the date.
- [ ] Do not try to "coach them through it" with meal plans alone. This is above your scope.

Script 14: Team Nutrition Policy Development

Context

You are helping a gym owner, school, or organization develop an internal nutrition policy — guidelines for how nutrition services are offered, what claims can be made, and how scope boundaries are maintained.

The Conversation

YOU

"Before we launch nutrition services, we need a clear policy document. This protects your coaches, your clients, and your business. Here's what it should cover."

YOU

"Section one: scope of service. Our coaches provide general nutrition guidance and customized meal planning. They do not diagnose conditions, prescribe therapeutic diets, or provide clinical nutrition therapy. This needs to be in writing, signed by every coach."

YOU

"Section two: referral protocols. Any client who presents with a medical condition requiring dietary management gets referred to a registered dietitian. We maintain a list of local RDs and we coordinate with them when needed. The referral doesn't mean we lose the client — it means we serve them properly."

YOU

"Section three: claims and marketing. No coach may claim that our nutrition services cure, treat, or prevent disease. Our marketing materials use language like 'supports fitness goals,' 'helps with body composition,' and 'builds sustainable eating habits.' Never 'fixes your metabolism' or 'cures inflammation.'"

YOU

"Section four: data privacy. Client dietary information is confidential. It is not shared with other staff, posted on social media, or discussed publicly without explicit written consent."

YOU

"Section five: documentation. Every client interaction related to medical disclosures, scope referrals, and allergy management must be documented in the client's file with the date and details."

Notes

- [ ] Have the policy document reviewed by a lawyer familiar with your jurisdiction's regulations on nutrition services.
- [ ] Update the policy annually or whenever regulations change.

Script 15: Medical Nutrition Coordination Scripts

Context

A client's doctor or registered dietitian has provided dietary recommendations that you need to integrate into the client's meal plan.

The Conversation

YOU

"I received the dietary guidelines from your [doctor/dietitian]. Let me walk you through how I'm incorporating them into your plan."

YOU

"Your [doctor/dietitian] recommended [specific guidelines — e.g., sodium below 2,000mg/day, potassium above 3,500mg/day, limited saturated fat]. I've rebuilt your plan with these constraints as non-negotiable filters. Every meal in your plan meets these parameters."

YOU

"What this means day-to-day: you'll notice [specific change — e.g., less red meat, more fish, lower-sodium seasonings, more potassium-rich foods like bananas and sweet potatoes]. The flavor profile is different, but I've selected recipes that work within the constraints without making your meals feel clinical."

YOU

"If your [doctor/dietitian] adjusts these recommendations at your next appointment, let me know immediately and I'll update the plan."

Notes

- ☐ Never override a doctor's dietary recommendations, even if you disagree with them.
- ☐ Keep a copy of the medical dietary guidelines on file.
- ☐ If a client is seeing both a doctor and an RD with conflicting recommendations, encourage them to have their providers communicate directly.

Script 16: Pediatric Nutrition Conversations

Context

A parent asks you to help with their child's nutrition. This requires extreme scope awareness — pediatric nutrition is heavily regulated and clinically complex.

The Conversation

YOU

"I'm glad you're thinking about [Child's Name]'s nutrition. Before I say anything, I want to be upfront: pediatric nutrition is a specialized field, and detailed dietary prescriptions for children fall outside my scope as a nutrition coach."

YOU

"What I can do is build family-friendly meal plans that support healthy eating for the whole household. If your child eats what the family eats — and the family is eating well — that's a great foundation."

YOU

"For anything specific to [Child's Name] — growth concerns, picky eating that goes beyond normal, weight management, food allergies that need clinical management, or sports nutrition for a young athlete — I'd recommend seeing a pediatric registered dietitian. They're trained in the specific nutritional needs of growing bodies."

YOU

"I'm happy to coordinate with them on the family plan so that what your household eats aligns with whatever they recommend for [Child's Name] individually."

Notes

- ☐ Never put a child on a calorie deficit or restrictive diet without explicit guidance from their pediatrician or pediatric RD.
- ☐ "Picky eating" in children is usually developmental and normal. If a parent is extremely concerned, refer to a pediatric feeding specialist.

Script 17: Geriatric Dietary Needs

Context

An elderly client (70+) or a caregiver asks about nutrition for an aging person. Concerns include muscle preservation, bone health, medication interactions, appetite loss, and chewing/swallowing difficulties.

The Conversation

YOU

"Nutrition in the later years has some unique considerations. Let me walk through what I'm adjusting in the plan."

YOU

"Protein is critical. After 70, the body's ability to synthesize muscle from protein decreases significantly. Your plan has a higher protein target — [X]g per day — spread across multiple meals. We're not trying to build bodybuilder-level muscle. We're trying to maintain the muscle you have, which is directly linked to independence, balance, and fall prevention."

YOU

"I'm also building meals around nutrient-dense, easy-to-prepare foods. If chewing or swallowing is a concern, let me know and I'll adjust textures — softer proteins, cooked vegetables, smoothies, soups."

YOU

"Medication interactions are something I take seriously. Please share your full medication list, and I'll cross-reference it with the foods in your plan. Some common interactions — like grapefruit with statins or leafy greens with warfarin — need to be managed through consistent intake rather than avoidance."

YOU

"If you have specific medical conditions affecting your diet — kidney disease, heart failure, diabetes — I'll need guidance from your doctor or dietitian before building the plan. I want to make sure everything is coordinated."

Notes

- ☐ Appetite loss is common in elderly clients. If caloric intake is dangerously low, refer to their physician.
- ☐ Hydration is often overlooked in older adults. Include reminders and hydrating foods.

Script 18: Oncology Nutrition Support

Context

A client is undergoing cancer treatment and asks about nutrition during chemotherapy, radiation, or recovery. **This is firmly clinical territory.**

The Conversation

YOU

"First, I want to say how much I respect you for focusing on nutrition during this time. It matters, and the fact that you're thinking about it shows real strength."

YOU

"I need to be completely honest with you: oncology nutrition is a specialized clinical field. The dietary needs during cancer treatment — managing nausea, maintaining weight, addressing taste changes, supporting immune function while potentially neutropenic — require guidance from an oncology-certified registered dietitian. This is not something I can safely coach you through on my own."

YOU

"What I can do: if your oncology RD provides dietary guidelines, I will build your meal plans around those guidelines precisely. I'm experienced with the EvoFit Meals platform and can accommodate any restriction or requirement they specify. I can also help with the practical side — meal prep, grocery lists, and making sure the food in front of you is something you can actually eat when appetite and taste are affected."

YOU

"Would you like me to help you find an oncology RD? I can also coordinate directly with them once you're connected."

Notes

- [ ] Never recommend supplements, herbal remedies, or "anti-cancer diets" to an oncology patient. Some supplements interact with chemotherapy drugs.
- [ ] If a client reports significant unintended weight loss during treatment, treat it as urgent and ensure their medical team is informed.

Script 19: Dialysis Diet Conversations

Context

A client on dialysis or with chronic kidney disease asks about nutrition. Renal diets are complex — they involve managing protein, sodium, potassium, phosphorus, and fluid intake simultaneously.

The Conversation

YOU

"Kidney nutrition is one of the most precise dietary situations there is. I need to be upfront: building a renal diet plan requires input from a renal dietitian who can work from your lab values."

YOU

"Here's what I need from you: permission to coordinate with your nephrologist or renal dietitian. They'll tell me your specific targets for protein, sodium, potassium, phosphorus, and fluid. Once I have those numbers, I can build meal plans that meet those targets while still including foods you enjoy."

YOU

"I won't guess on these numbers. Potassium and phosphorus management in kidney disease is clinical — getting it wrong can be dangerous. But once your specialist gives me the parameters, I can make the day-to-day eating experience much more enjoyable than the generic handouts most dialysis patients receive."

Notes

- ☐ Renal diets change as kidney function changes. Require updated lab-based targets every 1-3 months.
- ☐ Never recommend potassium-rich foods (bananas, potatoes, tomatoes) to a dialysis patient without confirming with their specialist.

Script 20: Bariatric Pre/Post Surgery

Context

A client is preparing for or recovering from bariatric surgery (gastric bypass, sleeve gastrectomy, or lap band).

The Conversation

Pre-surgery:

> **YOU**
> "Your surgeon has likely prescribed a pre-surgical diet — usually a liquid or low-calorie phase to shrink your liver before the procedure. My role is to make that phase as tolerable as possible. I'll build a plan that meets the surgical requirements while giving you as much variety as the guidelines allow."

Post-surgery:

> **YOU**
> "Post-bariatric nutrition goes through distinct phases — typically clear liquids, full liquids, pureed foods, soft foods, and then regular foods. Each phase has specific guidelines from your surgical team, and I'll follow those exactly."

> **YOU**
> "Once you're cleared for regular foods, your nutritional needs change permanently. Portion sizes are dramatically smaller. Protein becomes the absolute priority — your body can only absorb so much from each small meal, so every bite needs to count. I'll build plans with protein-first meals, easy-to-digest foods, and the specific supplementation your surgeon recommends."

> **YOU**
> "I'll need to coordinate with your surgical team throughout this process. They set the medical parameters; I handle the practical meal planning."

Notes

- ☐ Post-bariatric patients require lifelong supplementation (B12, iron, calcium, multivitamin). Their surgical team prescribes these; your job is to ensure the food plan supports absorption.
- ☐ Never encourage a post-bariatric client to eat more volume. Trust the surgical team's guidelines.

Script 21: Autoimmune Protocol Guidance

Context

A client with an autoimmune condition (Hashimoto's, lupus, rheumatoid arthritis, Crohn's, ulcerative colitis, psoriasis, etc.) asks about the autoimmune protocol (AIP) or anti-inflammatory eating.

The Conversation

YOU

"The autoimmune protocol is an elimination-style approach that removes potentially inflammatory foods and systematically reintroduces them to identify triggers. It's a well-structured framework, but I want to be clear: implementing AIP properly should involve your doctor or a registered dietitian who specializes in autoimmune conditions."

YOU

"Here's what I can do: if your specialist recommends AIP or a modified elimination protocol, I'll build your meal plans around those guidelines. The EvoFit Meals platform has filters for every major elimination category — nightshades, grains, legumes, dairy, eggs, nuts, seeds — so I can generate compliant plans efficiently."

YOU

"What I won't do is recommend you start AIP on your own without medical guidance. Elimination diets are restrictive, and for someone with an autoimmune condition, the nutritional balance needs to be carefully managed to avoid making things worse."

Notes

- ☐ AIP eliminates: grains, legumes, dairy, eggs, nuts, seeds, nightshades, refined sugars, alcohol, and food additives. It's highly restrictive.
- ☐ The reintroduction phase is where the real learning happens. Document every reintroduction and reaction.

Script 22: SIBO/Gut Health Conversations

Context

A client reports digestive issues — bloating, gas, constipation, diarrhea, food intolerances that seem to keep expanding — and asks if you can help with gut health or mentions SIBO (small intestinal bacterial overgrowth).

The Conversation

YOU

"Gut health is a big topic, and I appreciate you bringing it up. Let me separate what I can help with from what needs a specialist."

YOU

"If you're experiencing persistent digestive issues — especially if they're getting worse or if you're becoming intolerant to more and more foods — the first step is getting a proper diagnosis. SIBO, IBS, food intolerances, and other GI conditions require testing that only your doctor or a gastroenterologist can do."

YOU

"Once you have a diagnosis and dietary recommendations, I can build meal plans that accommodate them. Low-FODMAP plans, specific carbohydrate diets, elimination protocols — the platform supports all of these. I'll make sure your plan is compliant with your specialist's recommendations while still being food you want to eat."

YOU

"What I won't do is put you on a restrictive gut-health protocol based on symptoms alone. Without a diagnosis, we might eliminate the wrong things and make the situation worse."

Notes

- ☐ Low-FODMAP is the most evidence-based dietary approach for IBS but should be implemented with RD guidance.
- ☐ "Gut health" is a trend term that gets co-opted by unqualified practitioners selling supplements and protocols. Stay evidence-based.

Script 23: Sports Nutrition Periodization

Context

A serious athlete (competitive or recreational) needs their nutrition periodized — different nutritional strategies for different phases of their training cycle.

The Conversation

YOU

"Your training isn't the same intensity and volume year-round, so your nutrition shouldn't be either. We're going to periodize your eating to match your training phases."

YOU

"During your base/build phase, we focus on adequate fuel. Carbs are higher to support training volume. Protein stays consistent for recovery. Total calories are at or slightly above maintenance because you need the energy."

YOU

"During your peak/competition phase, we tighten things up. If you need to make weight or optimize body composition, we introduce a controlled deficit. Meal timing becomes more important — pre-training and post-training nutrition are dialed in precisely."

YOU

"During your off-season/recovery phase, we relax the structure. Calories come up, variety increases, and the psychological break from strict eating is just as important as the physical recovery."

YOU

"I'll build different plan templates for each phase. When you transition between phases, I swap the template. Your macros, your meal timing, and your food selection all shift to match what your body needs in that specific training block."

Notes

- ☐ For endurance athletes: focus on carbohydrate periodization (train-low, compete-high strategies).
- ☐ For strength athletes: focus on protein timing and caloric sufficiency.
- ☐ For team sport athletes: account for match-day nutrition, travel days, and recovery days within the weekly plan.

Script 24: Religious Dietary Accommodation

Context

A client follows religious dietary laws (halal, kosher, Hindu vegetarianism, Orthodox fasting, Ramadan, Lent, etc.) and needs their meal plan to accommodate these requirements.

The Conversation

> YOU
> "I want to make sure your plan fully respects your dietary practices. Can you walk me through the specific requirements so I get this right?"

For halal: "I'll ensure all meat in your plan is halal-certified. No pork or pork-derived ingredients. I'll flag any recipes that use alcohol-based extracts and provide alternatives." **For kosher:** "I'll separate dairy and meat meals, avoid non-kosher seafood and meats, and note which meals require specific preparation. If you follow strict kosher, let me know about your kitchen setup so I can plan accordingly." **For Ramadan/fasting periods:** "During Ramadan, I'll restructure your plan around suhoor and iftar. Suhoor will focus on slow-digesting foods — complex carbs, protein, healthy fats — to sustain you through the fast. Iftar will include a balanced meal with rehydration as a priority. Your macro targets stay the same; the timing shifts completely." **For Orthodox fasting:** "If you fast from animal products on certain days, I'll build those days with plant-based protein sources that hit your targets. The plan will alternate between fasting and non-fasting days automatically."

> YOU
> "Your religious practice is not an obstacle to your nutrition goals — it's a framework I build around. The EvoFit Meals library has extensive filtering options for halal, kosher, vegetarian, and vegan meals, so the recipe selection will be robust."

Notes

- [ ] Ask the client to describe their specific level of observance. "Halal" or "kosher" can mean different things to different people.
- [ ] During fasting periods, hydration and electrolyte management become critical. Adjust plans accordingly.

Script 25: Prenatal/Postnatal Nutrition

Context

A client is pregnant, trying to conceive, or in the postpartum period and asks about nutrition support.

The Conversation

Prenatal:

YOU

"Congratulations. Prenatal nutrition is incredibly important, and I want to support you — but I also want to be transparent about scope. Your OB/GYN or midwife should be the primary guide for your dietary needs during pregnancy, especially regarding caloric intake, specific nutrient requirements (folate, iron, DHA, calcium), and any pregnancy-related conditions like gestational diabetes."

YOU

"What I can do is build your meal plans around their recommendations. Once I know your caloric targets and any foods to avoid, I'll create plans that are nutrient-dense, manage nausea-friendly options for the first trimester, and adapt as your needs change each trimester."

YOU

"Foods I'll emphasize in your plan: folate-rich foods (leafy greens, legumes), iron sources (lean red meat, spinach), DHA-rich fish (salmon, sardines — avoiding high-mercury fish), and calcium sources. Foods I'll flag for avoidance: raw fish, unpasteurized dairy, deli meats, high-mercury fish, and excessive caffeine."

Postnatal:

YOU

"Postpartum nutrition is about recovery, energy, and — if you're breastfeeding — supporting milk production. This is not the time for aggressive calorie deficits. Your body just performed an extraordinary feat. We're going to nourish it."

YOU

"Your plan will be calorie-sufficient, protein-rich, and include foods that support lactation if you're breastfeeding. If you have weight loss goals, we'll address those gradually after your doctor clears you — typically 6-8 weeks postpartum, and even then, we'll proceed slowly."

YOU

"If you're experiencing postpartum mood changes, know that nutrition plays a role — omega-3s, vitamin D, and overall caloric sufficiency support mood regulation. But if you're experiencing significant postpartum depression or anxiety, please talk to your healthcare provider. I'll support you nutritionally, but the clinical side needs a specialist."

Notes

- [ ] Never recommend a calorie deficit for a pregnant client unless explicitly directed by their physician.
- [ ] Breastfeeding requires an additional 200-500 calories per day above pre-pregnancy maintenance.

Quick Reference: The Scope Cheat Sheet

TopicIn Scope (You Can Advise)Out of Scope (Refer Out) Macros and caloriesGeneral targets for fitness goalsTherapeutic diets for medical conditions Meal planningCustom plans for health and body compositionDiets prescribed for diabetes, kidney disease, etc. Food choicesRecommending whole foods, recipe suggestionsEliminating foods to treat allergies or medical conditions SupplementsGeneral fitness supplements (protein, creatine)Supplements to treat medical conditions Weight managementCalorie deficit/surplus strategiesWeight management for eating disorders HydrationGeneral hydration guidelinesFluid management for medical conditions Meal timingPre/post-workout nutritionTherapeutic fasting for medical purposes Emotional eatingRecognising patterns and suggesting structureDiagnosing or treating eating disorders Food intolerancesAvoiding reported intolerances in meal plansDiagnosing food allergies or intolerances Children's nutritionGeneral healthy eating guidance for parentsPaediatric dietary therapy PregnancyGeneral prenatal/postnatal meal planningGestational diabetes management, clinical prenatal nutrition Cancer/chronic diseasePractical meal support during treatmentTherapeutic diets for disease management Elderly nutritionProtein-focused plans, medication awarenessClinical geriatric dietary intervention **The golden rule:** If a client's question starts with "Can this food cure/treat/manage my [medical condition]?" — the answer is always "Let's get your doctor's input on that, and I'll build your plan around their recommendations." *This guide is part of the EvoFit Meals Business Vault. Access your full vault at meals.evofit.io/vault*

EvoFit Meals

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